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TABLE OF CONTENT

1. INTERFACING PRACTITIONERS WITH STUDENTS TO ACHIEVE APEX HEALTHCARE LEARNING ENVIRONMENT by Dr. Ahmed Tijjani Mora, Ph.D.; FPCPharm; FPSN; mni Dean, Faculty of Pharmaceutical Sciences, Kaduna State University (KASU), Kaduna, on the occasion of a Workshop for staff of Barau Dikko Teaching Hospital as a Tertiary Healthcare Provider at Hotel Seventeen, Lafia Road, Kaduna, on Wednesday, 13" April, 2016.
2. THE ROLE OF HEALTH REGULATORY AGENCIES IN ENHANCING EFFECTIVE HEALTHCARE DELIVERY SERVICES IN NIGERIA by Dr.Ahmed Tijjani Mora, B.Sc (Pharm); Ph.D; FPSN; FPCPharm; mni; on the occasion of 2016 Joint Scientific Conference and Exposition of the Association of Nigerian Physicians in the Americas (ANPA) and the Nigerian Association of Pharmacists and Pharmaceutical Scientists in the Americas (NAPPSA) at Ceasers Palace, Las Vegas, Nevada, USA on June 24, 2016.
3. THE TSAMIYA PHENOMENON: TRIBUTE TO MALLAM DAHIRU SULEIMAN WALI ON ATTAINING THE GREAT AGE OF 81 - By Dr. Ahmed Tijja'ni Mora, mni as tribute to Pharm. D.S. Wali, which appeared on. the Leadership Newspaper Edition of Saturday, July 23, 2016 on page 50, under TRIBUTE.

**"INTERFACING PRACTITIONERS WITH STUDENTS TO ACHIEVE APEX
HEALTHCARE LEARNING ENVIRONMENT"**

A Paper Presented

By

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On the occasion of a -

Workshop for staff of Barau Dikko Teaching Hospital as a tertiary
healthcare provider at hotel seventeen, Lafia road, Kaduna on
Wednesday, 13 April, 2016

Preamble

The Barau Dikko Teaching Hospital (BDTH), Kaduna is not the first to migrate from being a General Hospital (a secondary healthcare provider facility) to a Teaching hospital (a tertiary healthcare provider facility). Indeed, most teaching hospitals and/or tertiary healthcare institutions were once secondary healthcare providers.

Even before the establishment of the Faculty of Medicine, Ahmadu Bello

University (ABU/ Zaria in 1967, the Ahmadu Bello University Teaching Hospital (ABUTH), Zaria has been in existence since colonial days and it was known as Asibitin Kofar Doko. It was the facilities of this hospital later named General hospital, Zaria that were upgraded as ABUTH, Tudun Wada, Zaria before the present permanent structure at Shika which was officially commissioned on 4th November, 2005, some thirty (30) years since construction started. (Mahmoud H et al, 2012).

Similarly, the Federal Neuro-Psychiatric hospital, Kaduna was established by the defunct North Central State Government as a psychiatric, 100-bed hospital in 1975.

The Federal Government took over the hospital from the Kaduna State

Government in 1996, following the recommendations in 1994 of the National Committee for the Review of Psychiatric Hospitals and Services in Nigeria (FMOH, 2007).

The upgrading of the facilities of the former Barau Dikko Specialists Hospital to transform it to BDTH is therefore not unusual. With this upgrade, the institution will, in addition to offering healthcare services to patients now train healthcare personnel to perform professional health care services within the hospital as trainees and upon qualifying/ add to the much-needed human resources in health (HRH) to service the nation's health institutions at different tier- Primary, Secondary or Tertiary as the case may be.

In the case of BDTH its conversion by law is to effectively serve as the institution with the appropriate facilities and expertise for medical students of the Kaduna State University (KASU) to be trained during their clinical years of training presently and indeed for other courses whose students would require the facilities for their professional training as well.

It is from the point of view of this migration from being a secondary to a tertiary healthcare facility that BDTH as it is, will be discussed in this paper especially with the now new addition of students what! add to the hitherto only healthcare professionals and other staff and the patients and their relatives that transverse the corridors of the hospital. The additional work of "Teaching" in the name of the hospital says it all. The hospital staff need this kind of workshop in order to be re-oriented as to the new status, focus, aims and objectives of the hospital now as a teaching hospital of a state-owned University which is KASU.

Again, new cadre of staff will be recruited as part of the upgrade requirement to ensure proper teaching of the students is affected. Healthcare regulatory agencies such as the Medical and Dental Council of Nigeria (MDCN), the Pharmacists Council of Nigeria (PCN), the Medical Laboratory Science Council of Nigeria (MLSCN) will now assemble team with greater expertise to visit the hospital more often for inspection and accreditations of one healthcare professional programme or the other as the case may be due to the new tertiary status of the facility. For this too/ the conduct of staff; their skills and competencies in handling patients who⁷¹ now be received on referral basis from general hospitals within the state capital and beyond must be enhanced and this workshop provide one of such environments.

Vision and Mission of BDTH, Kaduna

Vision: To be a Tertiary Healthcare facility of highest ethical standard providing total healthcare training and services to our clients.

Mission: To offer highly specialized collaborative and integrated research and tertiary healthcare services and training that would meet the

healthcare expectation of our clients in a conducive setting that promotes hope/ motivation and dignity to mankind,

2.0 PROVISION OF HARMONIOUS LEARNING AND TEACHING ENVIRONMENT.

One of the greatest advantages in establishing BDTH as a teaching hospital is its presence within Kaduna metropolis / which has four (4) other public Specialist tertiary healthcare provider facilities as follows: -

- ❖ The Federal Neuro-Psychiatric hospital in Barnawa;
- ❖ The National Eye Centre by the western bye-pass, Kaduna;
- ❖ The National Ear Care Centre, just across the road and;
- ❖ The 44 Armed Forces Reference Hospital by the Sir Kashim Ibrahim House.

The emergence of BDTH as a fifth public tertiary healthcare facility in Kaduna metropolis will certainly elevate the quality of healthcare services to the populace at specialist level albeit on referral basis. Needless to add, there would be the much-expected collaboration and referral of patients across these tertiary healthcare provider facilities and more importantly, professional exchanges and advocacies in terms of workshops/ seminars and such other informed and learned meetings to sharpen skills and expertise of the staff. The greatest beneficiaries would be the students specially the medical students/ who could benefit from specialist knowledge from consultants in these four (4) other public tertiary healthcare Institutions that would, as time goes on be invited or be integrated within the BDTH teaching structure as per their specialized healthcare training as consultants, to the students due to proximity.

All healthcare professional practices entail long-life learning and as such the competencies, skills and expertise of the practitioners as specialists or consultants in the new teaching hospital must be sharpened from time to time to meet up with the dynamic nature of healthcare sciences. For instance, the defunct Institute of Health, ABU, Zaria that midwived the ABUTH to provide training environment for the medical students at inception was made up of the following constituent institutions:

- The Ahmadu Bello University Hospitals, Clinics and Dispensaries;
- The Ahmadu Bello University School of Nursing;
- The Ahmadu Bello University Medical Auxiliaries Training School;
- The Ahmadu Bello University School of Hygiene;
- Dala Orthopedic Hospital Kano with linkages to the Royal National Orthopedic Hospital, UK;
- General Hospital Malumfashi and associated Health Centres and Village Dispensaries; Guinness Eye Centre and the Maxillo-facial Unit.

These constituent units provided facilities for training personnel in the fields of medicine and health sciences while providing services and the medium for conducting research into health problems of the community.

It is necessary for BDTH to make research into various health sectors that affects the immediate community.

The availability of a library with relevant titles for the various practitioners to refer and consult to is needed. The teaching hospital must see itself automatically as a provider centre for Continuing Professional Development (CPD) programme for all the healthcare professions. It is not just enough for BDTH to operate as apex referral centre within the provision of the National Health Policy (NHP) alone, it must be a leading centre in the provision of data, information and expertise in all the health care professions. For instance, the Pharmaceutical Services Department should be equipped with state - of - the art equipment or instruments such that pharmacy students from KASU are not only taught practical cases in clinical pharmacy and pharmacy management in the hospital, but that other hospital pharmacy departments in the surrounding general hospitals should look up to the departments for guidance. The

various departments and units should be providing Bulletins for their various professional updates for the entire hospital community to benefit from. In other words, a village structure of healthcare professionals, supporting staff and students should evolve and work in harmony with

each other towards achieving the set goals and objectives of the teaching hospital.

3.0 PROFESSIONALISM AND WORK ETHICS.

A solid foundation of professionalism and work ethics is needed among the various healthcare personnel in the teaching hospital. All healthcare professionals in the teaching hospital should be role models for the students and serve as epitome of hope for the patients and patient's relations. As healthcare professionals, the staff members are binded by the following attributes:

- Expertise in specialized body of knowledge;
- A registration body;
- Possession of license to practice;
- Having been administered oath;
- Code of Conduct;
- Ethics of their various profession;
- Necessity for CPD to update professionalism and;
- Altruistic motivation for choosing to be healthcare professionals.

With these attributes, it means the expectations from our various regulatory and licensing agencies must always be prominent in our minds as we practice or teach the students or trainees placed before us. One of the best ways to live up to the expectations of our various regulatory agencies is drafting and observance of Standard Operating Procedure (SOPS) for the implementation of every major component of our professional services. In that way, professionals would have properly checked and armed themselves to ensure repetitive and system approach in handling every case.

Which are these regulatory agencies? They are as follows:

- ❖ Community Health Practitioners Registration Board of Nigeria;
- ❖ Dental Technologists Registration Board of Nigeria;
- ❖ Dental Therapists Registration Board of Nigeria;

- ❖ Health Records Officers Registration Board of Nigeria; Medical and Dental Council of Nigeria;
- ❖ Medical Laboratory Science Council of Nigeria;
- ❖ Medical Rehabilitation Therapists Registration Board of Nigeria;
- ❖ Nursing and Midwifery Council of Nigeria;
- ❖ Optometrists and Dispensing Opticians Registration Board of Nigeria;
- ❖ Pharmacists Council of Nigeria;
- ❖ Radiographers Registration Board of Nigeria;
- ❖ Institute of Chartered Chemists of Nigeria;
- ❖ Institute of Public Analysts of Nigeria.

4.0 QUALITY AND QUANTITY IMPROVEMENT OF LEARNING ENVIRONMENT.

One of the first major challenge for all the staff of the new teaching hospital is handling or relating with students. From the days of the old Nursing Home, to Sani Abacha Children hospital, to the New General hospital, to Barau Dikko Specialist hospital and now Barau Dikko Teaching

Hospital, staff members have always been by themselves alone and their patients and/or patient's relatives to contend with. Now with addition of students among the hospital community, there must be proper structure on how the professionals and even other non-professionals who do not teach relate with the students.

4.1 Quality:

Seeing and examining patients that hitherto takes few minutes will now take longer period due to the teaching nature of the practice. Students would have to be given the opportunity to review patients' medication, ask questions and be asked too and to document all observations.

4. 2 Quantity:

The operation at BOTH will now be a 24 hour, seven days a week continues activity with lots of documentation especially from one shift team to

another. Students will need to be allowed time and space to document findings. Statistics would have to be kept, data analyzed and findings shared. There will be more work, no doubt.

5.0 TEMPLATE FOR HARMONIOUS RELATIONSHIP BETWEEN' HEALTHCARE PROFESSIONAL STAFF AND MEDICAL AND PHARMACY STUDENTS.

The students from the Faculty of Medicine, KASU would practically be all over the teaching hospital once they've passed their pre-clinical years and their learning shifts to the teaching hospital. Pharmacy students would be present in the hospital in their 4th and 5th year for courses in Clinical Pharmacy.

- ❖ Dental Department;
- ❖ Ear, Nose and Throat Department; Other areas are as follows:
 - Laboratory for Haematology, Medical Microbiology, Chemical Pathology,
 - Morbid Anatomy and Forensic Pathology experiences.
- ❖ Radiology Department;
- ❖ The Wards to observe and review patients;

5.1 Medical Students:

The MB, BS students on clinical posting to BDTH will have to continually be in contact with the following departments or units:

- ❖ General Outpatient Department (GOPD);
- ❖ The operation theatres;
- ❖ Obstetrics and Gynaecology Department;
- ❖ Paediatrics Department;
- ❖ Ophthalmology Department;

The physiotherapy department to observe response of patients undergoing physiotherapy.

Pharmacy department to observe the compounding of medications and be informed of the Hospital formulary which should include different brands

of drugs and their strength and dosage forms. Social Welfare; Administration etc.

5.2 Pharmacy Students:

The B. Pharm. Students of KASU start their Clinical Ward rounds during their fourth (4th) year and essentially, their areas of linkages are as follows:

-

Pharmacy Department of the teaching hospital where they would be expected to observe and partake in dispensing of prescribed drugs to patients and how they counsel them. They should have proper understanding of the computerized dispensing system as well as Compounding of some extemporaneous preparations on a large scale. Students should examine prescriptions critically to avoid drug-drug, food-drug, and drug-diagnostic reagent interactions and avoid medication error generally.

Their ward round is to ensure that the prescribed drug for the inpatients, duly dispensed through the ward nurse is acting in the patient and giving the desired result as expected. Side effects and patient compliance will be monitored and recorded. There must be harmony between the matron in charge of the Wards and the hospital pharmacist, along with Faculty of Pharmaceutical Sciences lectures that will both serve as preceptors to guide and teach the students in the wards. Concepts like the unit dose dispensing, the Drug Revolving Found (ORF) Scheme operation as well as entries for narcotics, dispensed drugs for patients enrolled for the National Health Insurance Scheme (NHIS) that the students have been lectured the theory aspect would now be practicalised in the teaching hospital.

5.3 The Nursing Care Personnel

The services of the nurse are crucial in any teaching hospital, as the services of the nursing care personnel goes beyond patient care, drug administration and management of the wards. The nurses should provide the environment within the teaching hospital for effective learning and teaching, most of which is in the wards. Attitudes of the medical and

pharmacy students and the doctors and pharmacists who takes them round would have to be moderated to confer respect to the Wards and the patients as being managed by the nurses. Entry into the wards would have to be timed at a particular part of the day to ensure that certain categories of patients are not disturbed. In any ward round, the nursing personnel of the Ward must be present and participate in the review of the patient by way of offering valuable information about the patient to the medical or pharmacy teams at any point in time.

6.0 CONCLUSION, RECOMMENDATIONS AND IMPLEMENTATION

STRATEGIES:

The teaching of medical courses goes hand - in - hand with the establishment of a teaching hospital, hence the need to upgrade the facilities at the former BDSH, from being of secondary healthcare provider status to BDTH, a tertiary healthcare institution to enable the MB. BS students of Kaduna State University (KASU) an environment for effective learning and practice during their clinical years of training. What took place in ABU, Zaria in 1967 with the establishment of a Faculty of Medicine and closely followed by that of Pharmaceutical Sciences in 1970 appears to be repeating itself in KASU with Faculties of Medicine and Pharmaceutical Sciences established in 2009(?) and 2012 respectively.

The teaching component in the aims, objectives and indeed mission of BDTH makes all the difference in the way and manner the philosophy of provision of healthcare services is provided to members of the public henceforth. The presence of healthcare professionals as staff in BDTH and whose regulatory agencies were established with a sound foundation to control and regulate their various education and practices again emphasized the need for a conducive environment for teaching, learning, research and healthcare delivery of tertiary nature.

Similarly, the life-long learning mantra associated with every healthcare profession by way of practitioners subjecting themselves periodically to Continuing Professional Development (CPD) programmes appropriate to their specialisation again necessitate the emphasis of periodic advocacies as

in conferences, workshops, seminars and the likes by the different professionals to enhance their skills, expertise and competencies, since BDTH has now been upgraded to a tertiary public healthcare provider facility.

Fourthly, the presence of at least four (4) other specialist and/or tertiary public healthcare facilities within the state capital is a big motivation for BDTH to initiate collaborative discourse with them at inter - professional intellectual basis in addition to the obvious horizontal rapport that will be expected among the five (5) public healthcare institutions within the state capital.

More to the point, the medical and pharmacy students of KASU who would be the direct beneficiaries in the teaching hospital should count themselves very lucky, compared to their counterparts elsewhere in the country.

There is hardly a city in Nigeria with the possible exception of Lagos where one can find concentration of consultants or specialists in various and diverse healthcare fields such as in Eye, Ear, neuro-psychiatry and in trauma (as in Armed Forces reference hospital) as in Kaduna. If one extends it on a state- wide basis, we have the National Tuberculosis and Leprosy Centre in nearby Saye, and of course the Federal Government owned conventional University teaching hospital in Shika all in nearby Zaria just an hour drive from Kaduna. There are certainly areas for teaching and [earning in terms of collaboration across tertiary/specialists healthcare levels and vertically on referral basis from the multitude of general hospitals that are secondary healthcare provider facilities within the state capital.

In conclusion, I will round up this presentation with some recommendations and implementation strategies as follows:

6.1 Recommendation Number One:

The Barau Dikko Teaching Hospital (BDTH) should fully exploit the presence of the four (4) other tertiary/specialist public healthcare provider facilities within Kaduna to improve skills, expertise and competencies of its

healthcare professionals' staff which would ensure the imparting of the appropriate advancements in the various specialist areas of tertiary healthcare services to the medical and/or pharmacy students of KASU.

6.1.1 Implementation strategies:

The management of the teaching hospital to organize periodic seminars, conferences and workshops addressing topical subjects of contemporary healthcare importance along with the four (4) other tertiary/specialist public healthcare provider facilities within Kaduna and to be attended by all professionals as well as medical and pharmacy students of KASU; BOTH as the only conventional University teaching hospital among the tertiary/specialist public healthcare provider facilities within Kaduna to take the lead as the key provider centre for all manner of Continuing Professional Development (CPD) programmes for the healthcare professions within the metropolis to benefit from on a regular basis;

The teaching hospital to establish a healthcare journal where research studies from the four (4) other public healthcare teaching/specialist facilities and BDTH itself would be published periodically.

6.2 Recommendation number Two:

The management of the teaching hospital should provide an enabling environment to fully integrate and accommodate the medical and pharmacy students that would now, for the first time in the more than thirty (30) years of the existence of the hospital (as secondary healthcare facility) be additional cadre within the hospital community.

6.2.1 Implementation strategies:

All healthcare professionals regulated by any of the thirteen (13) healthcare regulatory agencies in the country and who are presently under the employment of BDTH to be given some teaching allowances; since beside their specializations duties as healthcare professionals, they will be expected to teach medical and / or pharmacy students of KASU in respect of aspects of their professional practices.

Adequate lecture rooms to be provided within the hospital to enable students a place to stay in between lectures and ward rounds to avoid wandering into units or departments that may not be ready to receive them; The Wards that is the "main" theatre for teaching the students to be accessible to the later at specific times in order to avoid infringement. Upon the work of the ward nurses or patients rest period; A cafe, restaurant, and common room to be established in order to enable both staff and students to interact and socialize outside of units or departments where professional practices take place.

6.3 Recommendation number three:

The management of the teaching hospital should equip a modern library area with the latest ICT gadgets to ensure access to updated healthcare information at all times for both staff and students.

6.3.1 Implementation strategies:

An electronic library with modern and up-to-date facilities to be installed to aid easy access to relevant healthcare information at all times; A Drug Information Centre (DIC) to be linked to all departments to ensure easy retrieval of information on drugs by both staff and students for the benefit of the patients and for research; Attendance to national and international conferences within Nigeria and abroad to be encouraged through periodic funding of budget code for such.

Thank you all for listening

Dr. Ahmed T. Mora

Wednesday, 13" April, 2016

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**"THE ROLE OF HEALTH REGULATORY AGENCIES IN ENHANCING
EFFECTIVE HEALTH CARE DELIVERY SERVICES IN NIGERIA"**

Being Text of a Paper Presented

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**ON THE OCCASION OF 2016 SCIENTIFIC CONFERENCE AND
EXPOSITION OF THE NIGERIAN ASSOCIATION OF PHARMACISTS AND
PHARMACEUTICAL SCIENTISTS IN THE AMERICAS (NAPPSA)**

AT LAS VEGAS, NEVADA, USA

June 23- 26, 2016

1.0 Preamble

The history of regulating healthcare services in Nigeria dates back to 1927, when the Board of Medical Examiners (BME) was established by the British Colonial administration. From the BME, other regulatory agencies evolved at different times to the present thirteen (13) health regulatory agencies that cater for the control and regulation of one healthcare profession or the other and as agencies of the Federal Government under the supervision of the Federal Ministry of Health (FMOH).

The responsibilities of these health regulatory agencies under the leadership of their various Registrars are similar even though the healthcare focus of each of the professions is different.

Essentially, they regulate and control the education and practice of these healthcare professions at the Primary, Secondary, and Tertiary healthcare provider centres responsible for the provision of Primary healthcare,

Secondary healthcare and Tertiary healthcare services in public health institutions, respectively.

Similar control and regulation also take place in all private health care provider facilities. The health regulatory agencies do more. They ensure the competencies of the healthcare professionals are enhanced as per their skills, expertise and competencies through mounting Continuing Professional Development (CPD) programmes in accredited centres and allotting points to ensure that these healthcare professionals are up-to-date in the knowledge of their various professions for the interest of their patients, students and fellow healthcare colleagues to benefit from. This Paper examines four (4) of such healthcare regulatory agencies, notably the Health Record Officers Registration Board of Nigeria (HRORBN), the Pharmacists Council of Nigeria (PCN); the Medical and Dental Council of Nigeria (MDCN) and the Dental Technologists Registration Board of Nigeria (DTRBN). A questionnaire was designed and administered to the four (4) agencies; and was completed, retrieved and analysed.

2.0 The Role of Health Regulatory Agencies in Advancing

Healthcare Delivery Services in Nigeria

The renowned medic and academic, Professor Emeritus Umaru Shehu, FAS stated as follows in respect of the roles of professional regulatory bodies

The roles of professional regulatory bodies are usually well defined and they are more or less the same all over the world. They include the following:

- (i) Setting minimum entry qualifications for students aspiring to Join the profession;
- (ii) Approving the curricular for training in the various professions;
- (iii) Monitoring the training of students to ensure compliance with all the regulations;
- (iv) Certification and re-certification of programmes;

- (v) Facilitating Continuing Professional Development (CPD) programme for practitioners of the profession and;
- (vi) Ensuring compliance with professional ethics and morality in practice (CRHP, 2008).

2.1 The Establishment of the Committee of Registrars of the Health Professions (CRHP)

The date, August 19, 2004, will forever be a special one to members of the healthcare professions in Nigeria and indeed the West African region. It was on that day, that the then Honourable Minister of Health, Professor Eytayo Lambo, inaugurated the CRHP. The objectives of the CRHP are as follows:

- (i) Raise the level of ethical conduct in healthcare delivery services in Nigeria;
- (ii) Set targets for efficient performance/
- (iii) Facilitate Continuing Professional Development (CPD) for the Health Professions in Nigeria;
- (iv) Enlighten the general public on health matters;
- (v) Harmonise all regulatory activities to ensure efficiency and effectiveness in the performance of the various functions of the diverse professionals in the health sector;
- (vi) Act as a platform for achieving synergy of the various healthcare delivery efforts of government by harnessing the peculiar and unique strength of the various professionals/ and
- (vii) Foster unity and mutual respect among the health professions in Nigeria.

While inaugurating the CRHP, the Honourable Minister made the following statement, among others:

It is, therefore, heart-warming that the Registrars of all the regulatory bodies in the health sector are coming together under one umbrella. This is commendable and should be applauded. I want to urge you to use this committee as a springboard to rejuvenate the team spirit that is fast fading away (Lambo, 2004).

These highly encouraging but weighty words were re-echoed by Dr. A. A. Ibrahim, the present Registrar of the MDCN and current Chairman of the CRHP, when he stated, while delivering a paper, as the Keynote Address Speaker in Kaduna, on the occasion of Sensitisation Workshop for Staff of Barau Dikko Teaching Hospital (BDTH) of Kaduna State University (KASU) on becoming a tertiary healthcare provider facility, as follows:

...I must confess that the most single issue, that is threatening to overwhelm every investment that has been made in the healthcare delivery system in Nigeria is not always about the availability of facilities for healthcare delivery, but the issue of inter-professional rivalry among the health professions... (Ibrahim, 2016).

3.0 The Role of Health Regulatory Agencies in Enhancing Effective Healthcare Delivery Services in Nigeria: A Study of HRORBN; PCN; MDCN and DTRBN

The parameters compared were as follows:

- (A) Year of establishment;
- (b) Applicable Law of the Federation of Nigeria (LFN) establishing the Agency;
- (c) Location of the Head office/ Registry;
- (d) Staff strength as at January 31, 2016;
- (e) Basic Responsibilities:
 - (i) Accreditations;
 - (ii) Inspections;

- (iii) Examinations;
 - (iv) Preparations of Benchmark Minimum Academic Standards (BMAS);
 - (v) Conducting Continuing Professional Development (CPD) programmes;
 - (vi) Preparations of Minimum Standards of Practice;
 - (vii) Registration and Licensing of Professionals;
 - (viii) Registration and Licensing Practice Premises;
 - (ix) Capacity building programmes for staff;
 - (x) Others (Specify).
- (f) Ranking of where most of the Agency's regulatory functions take place –
- (i) University;
 - (ii) Tertiary Healthcare facilities;
 - (iii) Secondary healthcare facilities;
 - (iv) Primary healthcare facilities;
 - (v) Private Practices;
 - (vi) Schools for Health Technologies;
 - (vii) Institutions for the training of the regulated professional cadre;
 - (viii) Missionary health Institutions;
 - (ix) Ministries of Health;
 - (x) Others (Specify)
- (g) Ranking in percentages collaboration with sister healthcare regulatory bodies

- i. Community Health Practitioners Registration Board of Nigeria (CHPRN);
- ii. Dental Technologists Registration Board of Nigeria (DTRBN);
- iii. Dental Therapists Registration Board of Nigeria (DTRBN);
- iv. Health Record Officers Registration Board of Nigeria (HRORBN);
- v. Institute of Chartered Chemists of Nigeria (ICCON);
- vi. Institute of Public Analysts of Nigeria (IPAN);
- vii. Medical and Dental Council of Nigeria (MDCN);
- viii. Medical Laboratory Science Council of Nigeria (MLSCN);
- ix. Medical Rehabilitation Therapist Board Nigeria (MRTBN);
- x. Nursing and Midwifery Council of Nigeria (NMCN);
- xi. Optometrists and Dispensing Opticians Registration Board of Nigeria (ODORBN);
- xii. Pharmacists Council of Nigeria (PCN);
- xiii. Radiographers Registration Board of Nigeria (RRBN).

(h) Percentage ranking of challenges faced by the Healthcare regulatory bodies: -

- i. Lack of personnel;
- ii. Poor funding;
- iii. Lack of adequate logistics;
- iv. Unsuitable office accommodation;
- v. Interference by the supervising Federal Ministry of Health (FMOH)
- vi. Lack of training and development of registry staff;
- vii. Interference by the professional Association;
- viii. Infighting among registry staff;
- ix. Lack of staff motivation;
- x. Others.

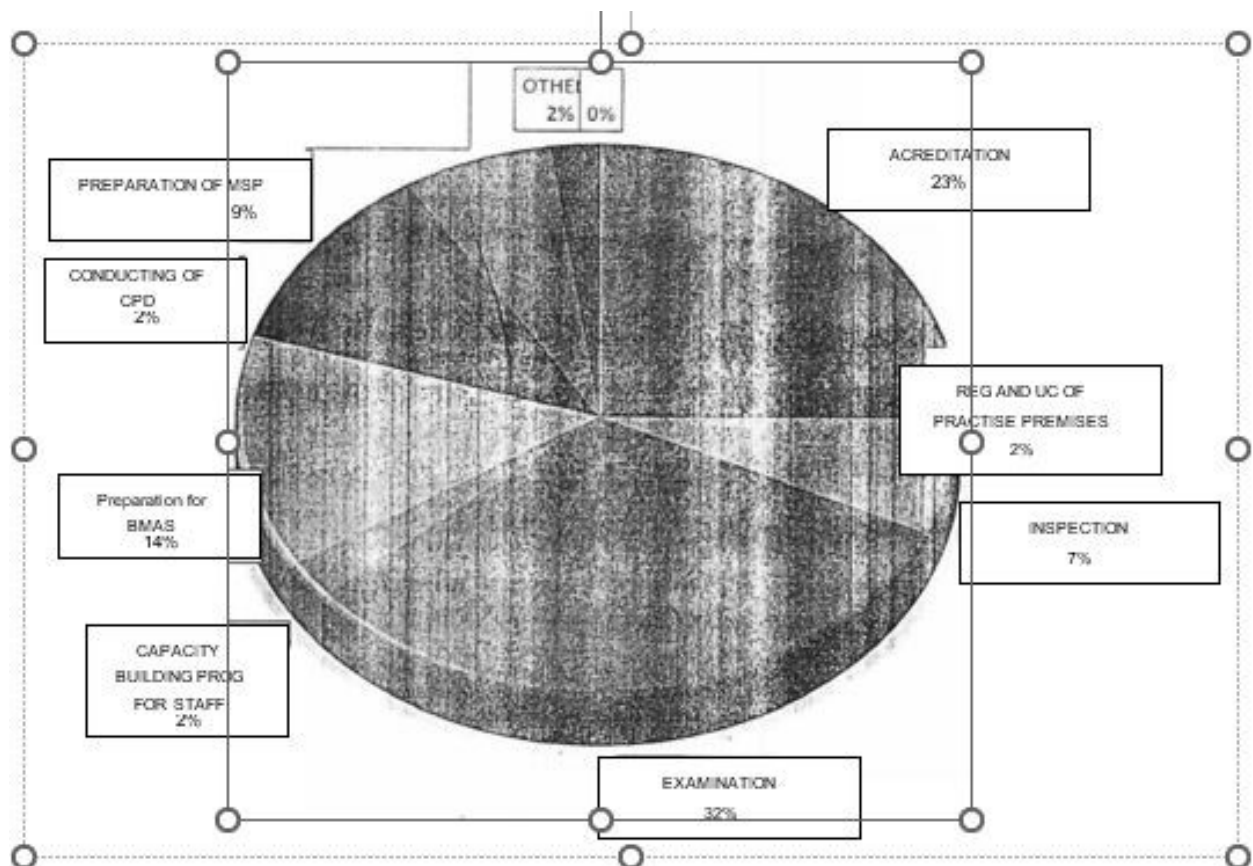
3.1 THE HEALTH RECORD OFFICERS REGISTRATION BOARD OF NIGERIA (HRORBN)

The Board was established in 1989 and the establishment was applicable to

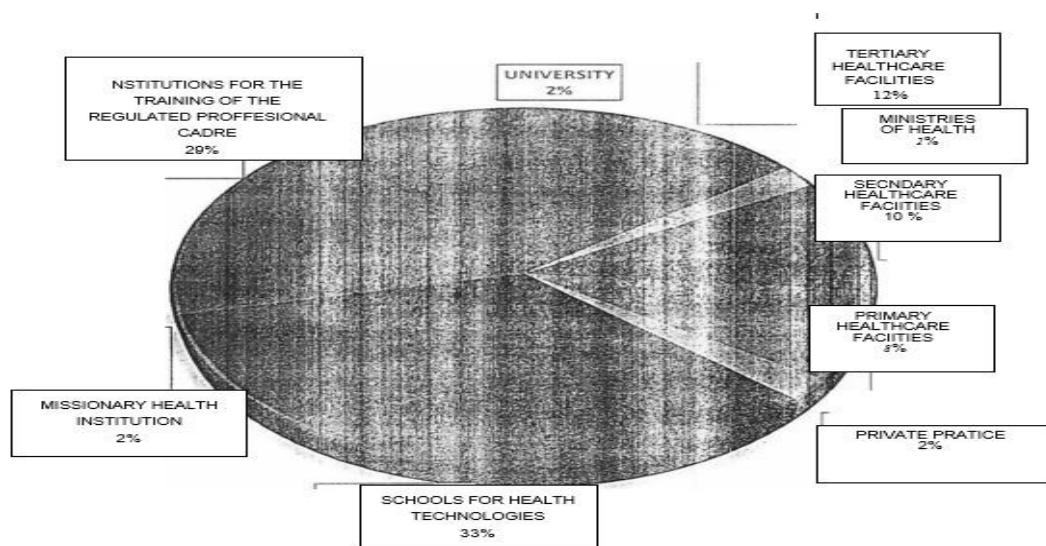
DECREE 39,1989 (CAP 166 OF 1990 LFN) with its head office located at 16, Kinshasa Street, Wuse, Zone 6, Abuja. The Board has total staff strength of 137 staff.

(a) Percentage score of the Board's Responsibility to the listed

Functions Interpretation According to the respondent rating of the Board's responsibility to the listed functions, it is responsible for:



3.1 (b) Places Where the Board's Regulatory Activities often takes Place.



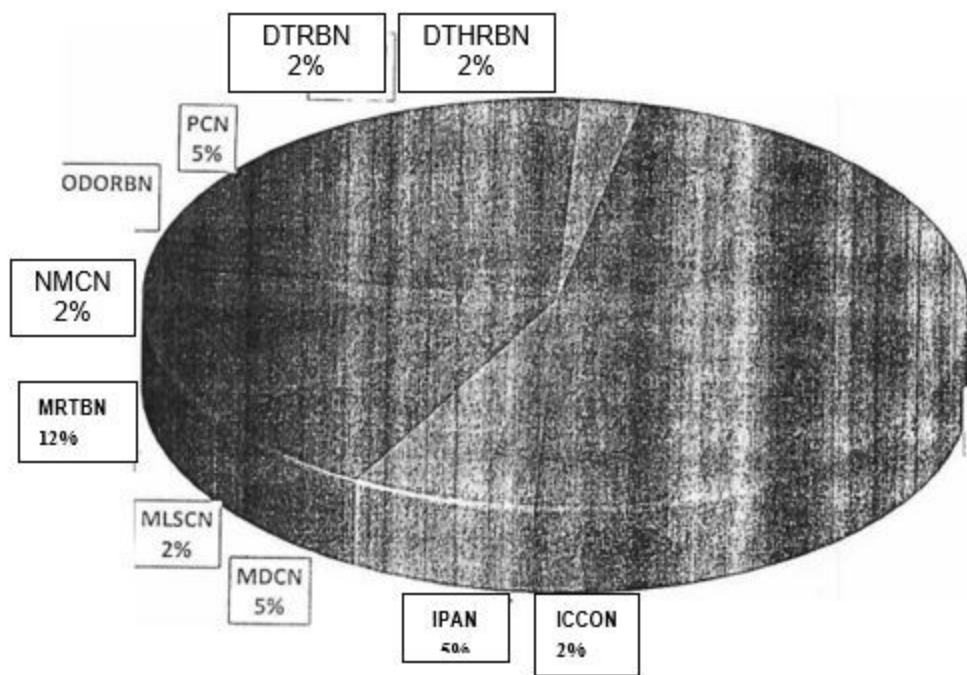
Interpretation

According to the respondent rating, the Board's regulatory functions take place in the following places (with percentage frequency);

- i. 33% on Schools for Health Technology
- ii. 29% on institution for the training of the regulated professional cadre Hence, the major places where the Boards regulatory functions take place are at Schools of Health Technologies and institutions for training the regulated professional cadre and healthcare facilities, among others.
- iii. 29% Poor funding
- iv. 24% Interference and the duplication of regulatory roles

Hence, we can conclude that the Council's major challenges are poor funding, interference and duplication of regulatory roles and lack of personnel respectively, among others.

3.1 (C) Sister Regulatory Bodies that the Board often Collaborates with.



Interpretation

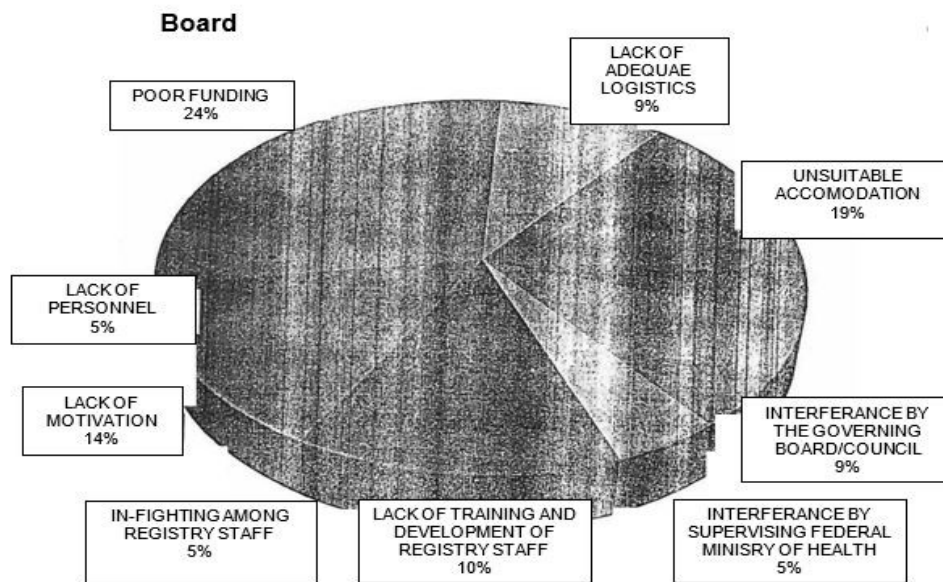
According to the respondents, the sisters' bodies the Board often collaborates with (together with their frequency of collaboration) are:

- i. 12% with the Medical Rehabilitation Therapist Board of Nigeria (MRTBN)
- ii. 10% with the Community Health Practitioners Registration Board of Nigeria (CHPRN).

As such, we conclude that the Board collaborates more with the Medical Rehabilitation Therapist Board of Nigeria (MRTBN) and the Community Health Practitioners' Registration Board of Nigeria (CHPRN) respectively, among others.

The Board/Council/Institute enjoys a cordial relationship with other professional associations.

3.1 (d) Percentage Rating of the Challenges Faced by the



Interpretation

Challenges faced by the Board (with their percentage rating) are:

- i. 24% Poor funding
- ii. 19% Unsuitable office accommodation
111. 10% Lack of training and development of registry staff

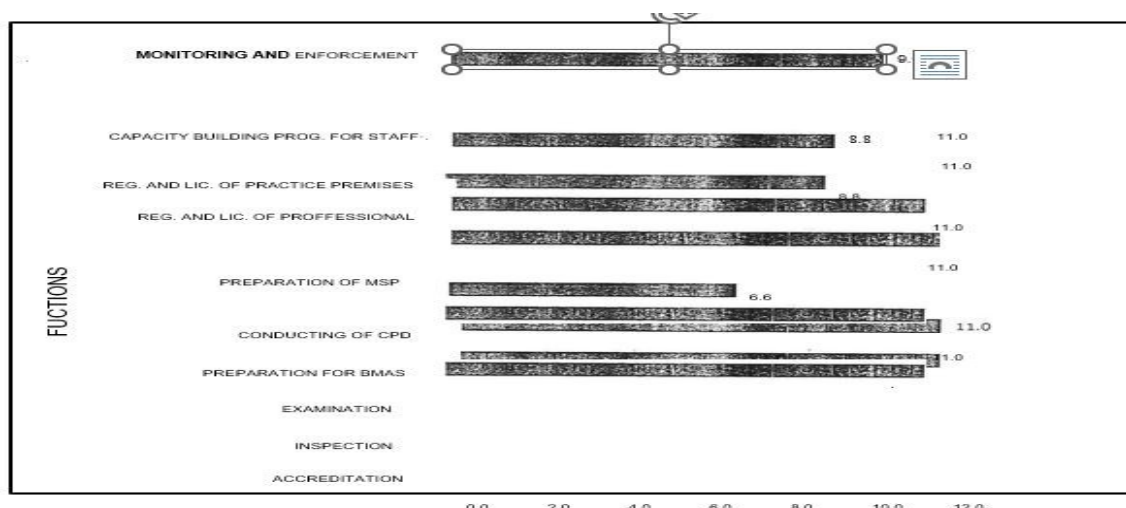
Hence, we can conclude that the Board's major challenges are poor funding, unsuitable office accommodation and lack of staff motivation respectively, among others

3.2 THE PHARMACIST'S COUNCIL OF NIGERIA (PCN)

The above Council was established in 1936 and the establishment

was applicable to Decree 91 of 1992 with a head office located at Plot 7 /9 Idu Industrial Layout, Idu, Abuja. The Council has total staff strength of 282.

3.2a. Percentage Score of the Council's Responsibility to the Listed Functions.



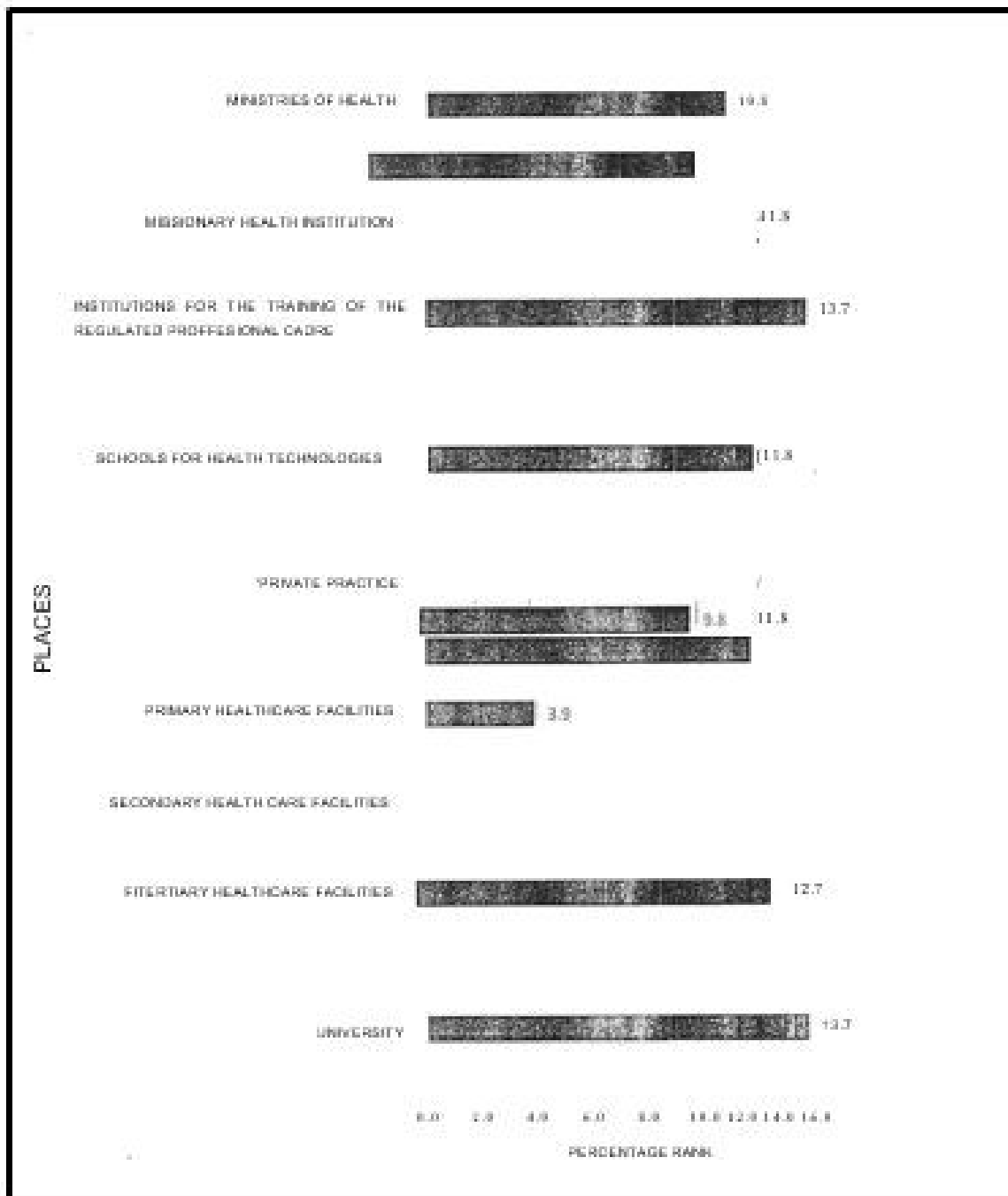
Interpretation

According to the respondent rating the Council is responsible for

- i. 11 % accreditation.
- ii. 11 % inspection.
- iii. 11 % preparation of benchmark minimum academic standards (BMAS)
- iv. 11 % conduction of continuing professional development (CPD) programmes
- v. 11 % preparation of minimum standards of practice
- vi. 11 % registration and licensing professionals.
- vii. 11 % registration and licensing practice premises.

Hence, the Council performs majorly the functions of accreditation, inspection, preparing benchmark minimum academic standards (BMAS), conducting continuing professional development, registration and licensing professionals and practice premises, among others.

3.2b. Places where the Council's Regulatory Activities often take place.



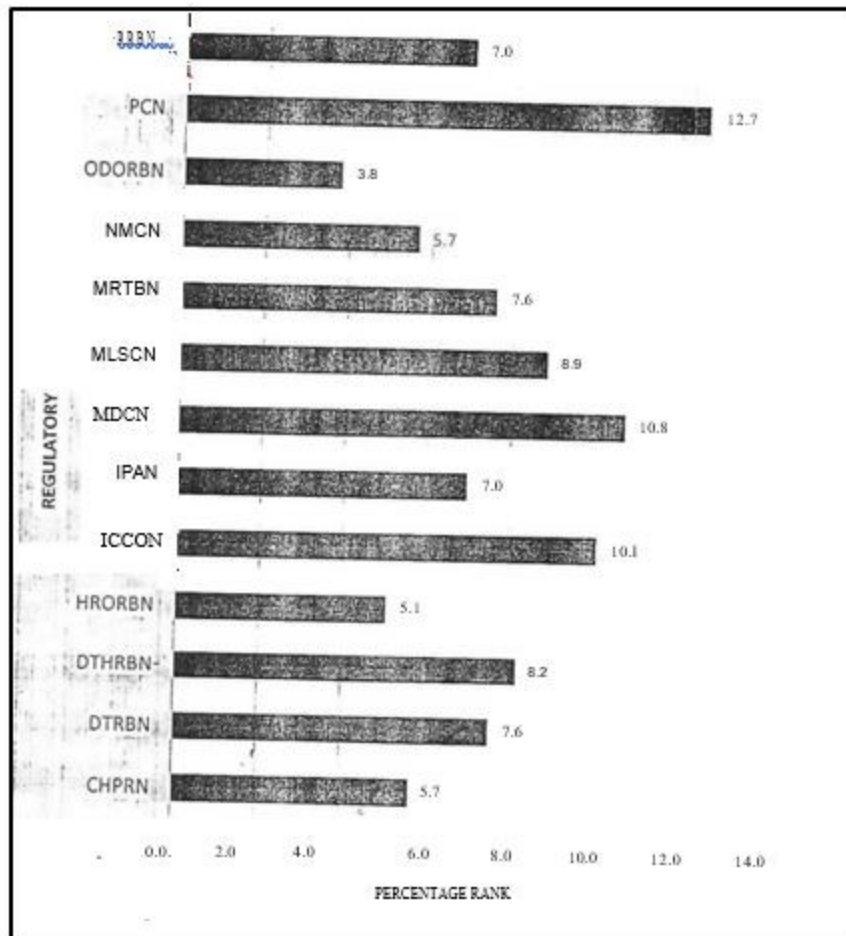
Interpretation

According to the respondent rating, the Council's regulatory functions take place in the following places (with percentage frequency):

- i. 13.7% on university grounds
- ii. 13.7% on institution for the training of the regulated professional cadre

Hence, the Council uses all the above listed places majority for their regulatory functions, except for primary health facility with a 3.9% usage.

3.2c. Sister regulatory Bodies that the Council Collaborates with



Interpretation

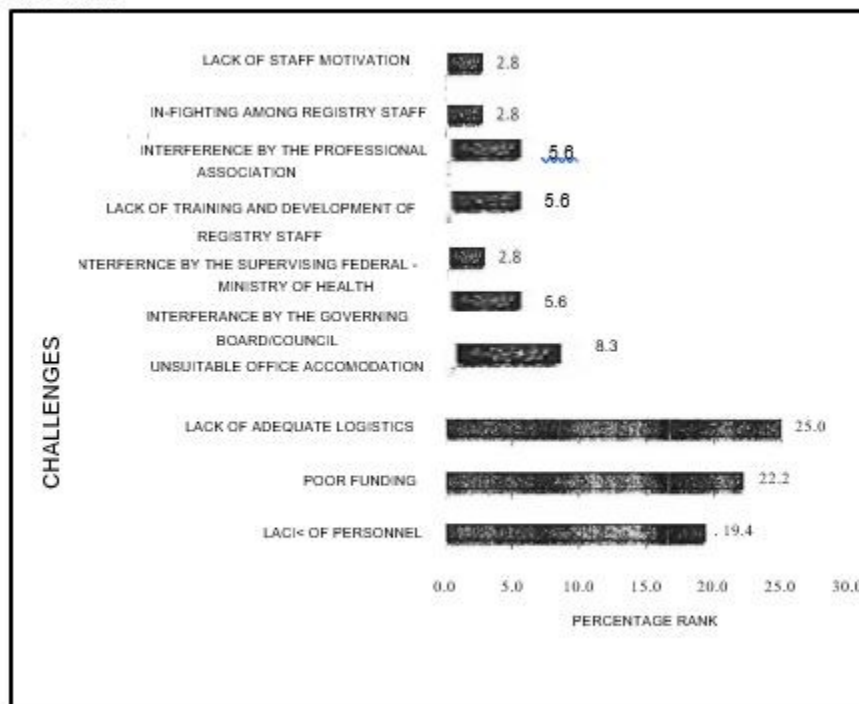
According to the respondents, the sisters' bodies the Council often collaborates with (together with their frequency of collaboration) are:

- i. 9.9% with the Institute of Chartered Chemists of Nigeria (ICCON)
- ii. 10.8% with the Medical and Dental Council of Nigeria (MDCN)
- iii. 8.9% with the Medical Laboratory Science Council of Nigeria

As such, we conclude that the Council collaborates with all the listed bodies effectively and efficiently.

8.0 The Council enjoys a very cordial relationship with other professional associations.

3.2d. Percentage Rating of the Challenges Faced by the Council.



Interpretation

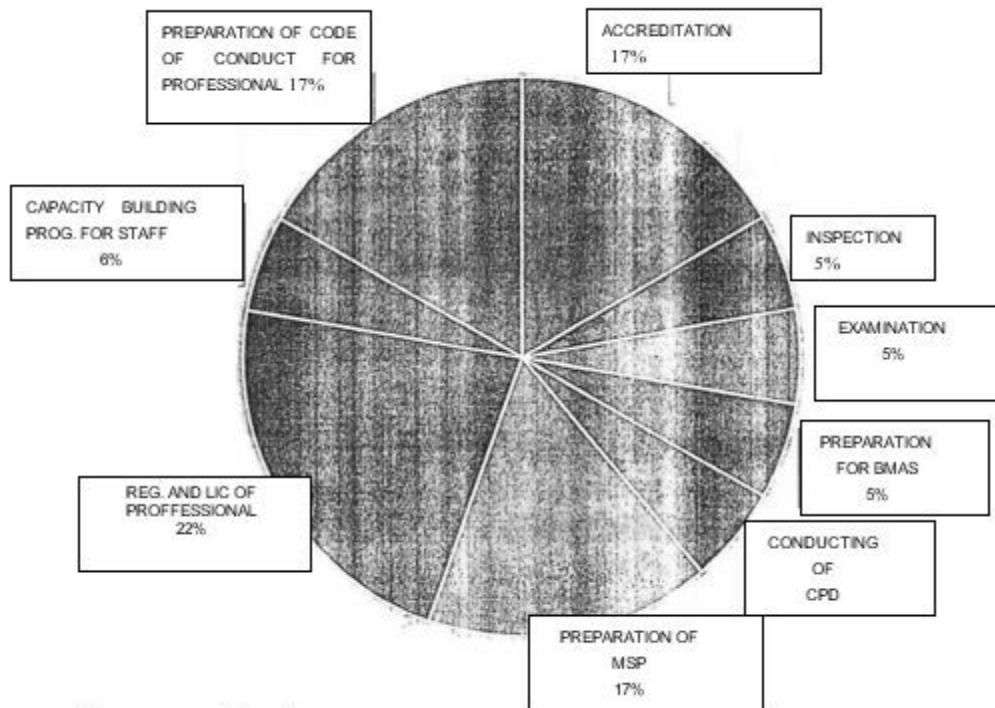
The challenges faced by the Council (with their percentage ratings) are:

- i. 19.4% Lack of personnel
- ii. 22% Poor funding
- iii. 25% Lack of adequate logistics

Hence, we can conclude that the Council's major challenges are lack of adequate logistics, poor funding and lack of personnel, respectively.

3.3 THE MEDICAL AND DENTAL COUNCIL OF NIGERIA (MDCN) The above council was established on 18th December, 1963 and the establishment was applicable to CAP MS LFN 2010. The council has total staff strength of 121 staff.

3.3a. Percentage Rank of the Council's Responsibility to the outlined functions.

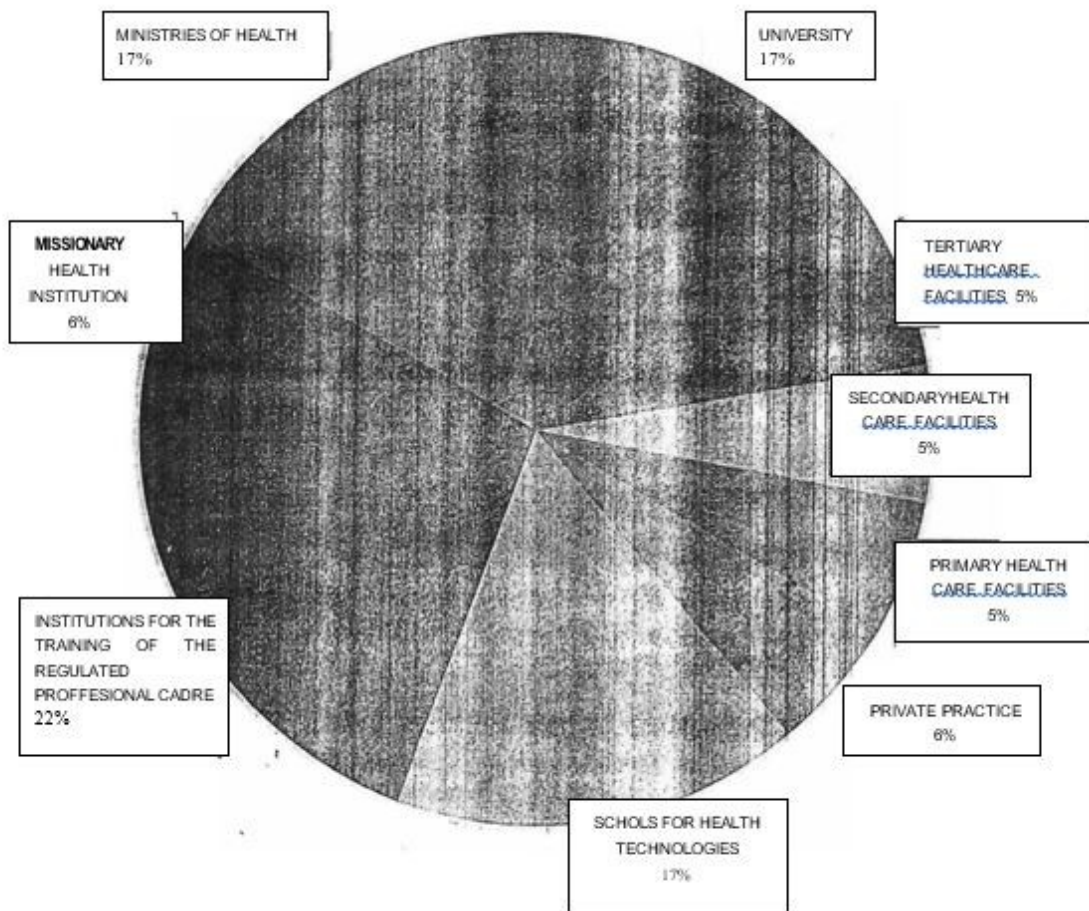


Interpretation.'

According to the respondent rating, the Council is responsible for

- i. 17% accreditation.
- ii. 22% registration and licensing of professionals.
- iv. 17% preparation of code for professions.
- v. Hence, the Council performs majorly the function of the registration and licensing of professionals, disciplining members and the preparation of the code of conduct for the professionals, among others.

3.3b. Percentage Rank of the Places where the Council's Regulatory Activities takes place.



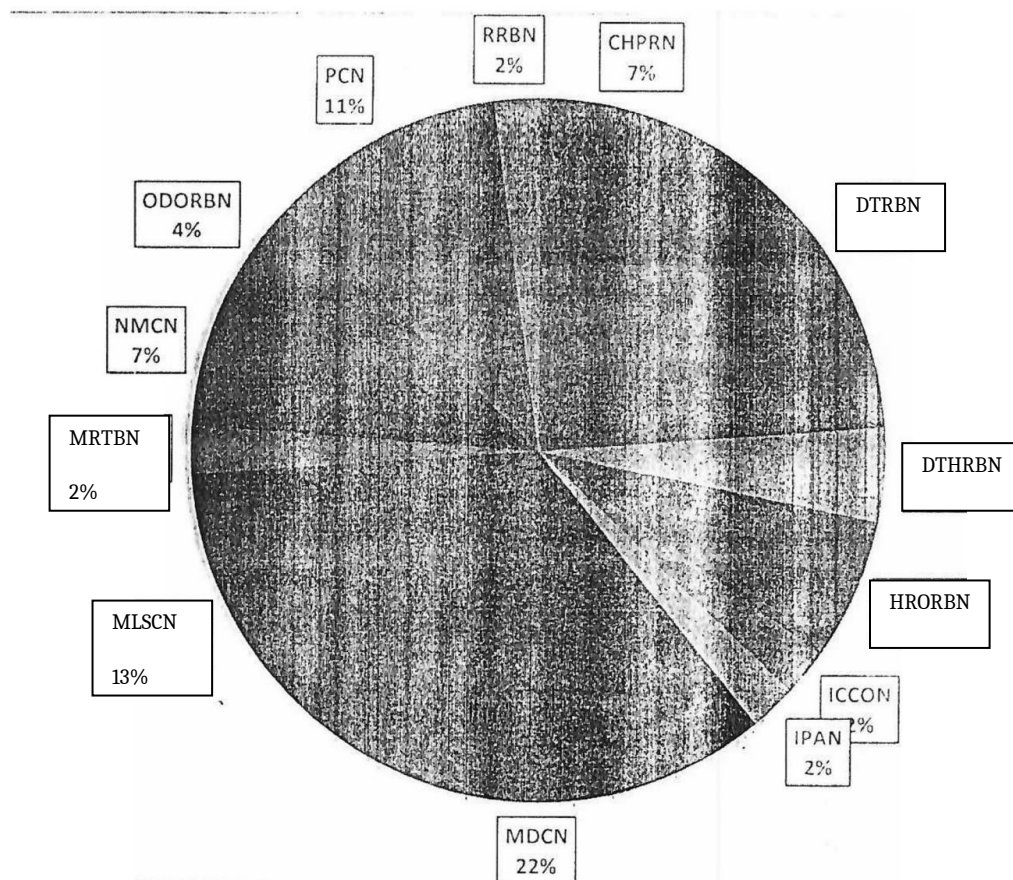
Interpretation

According to the respondent rating, the Council's regulatory functions take place in the following place (with percentage frequency):

- i. 17% on university grounds
- ii. 25% on tertiary health care facilities
- iii. 17% schools of Health Technologies (SHTs)

Hence, the Council uses universities and the institutions for the training of the regulated professional cadre majorly for their regulatory functions.

3.3 (c) MDCN do not collaborate with any sister regulatory body. Hence, they operate in isolation.



Interpretation

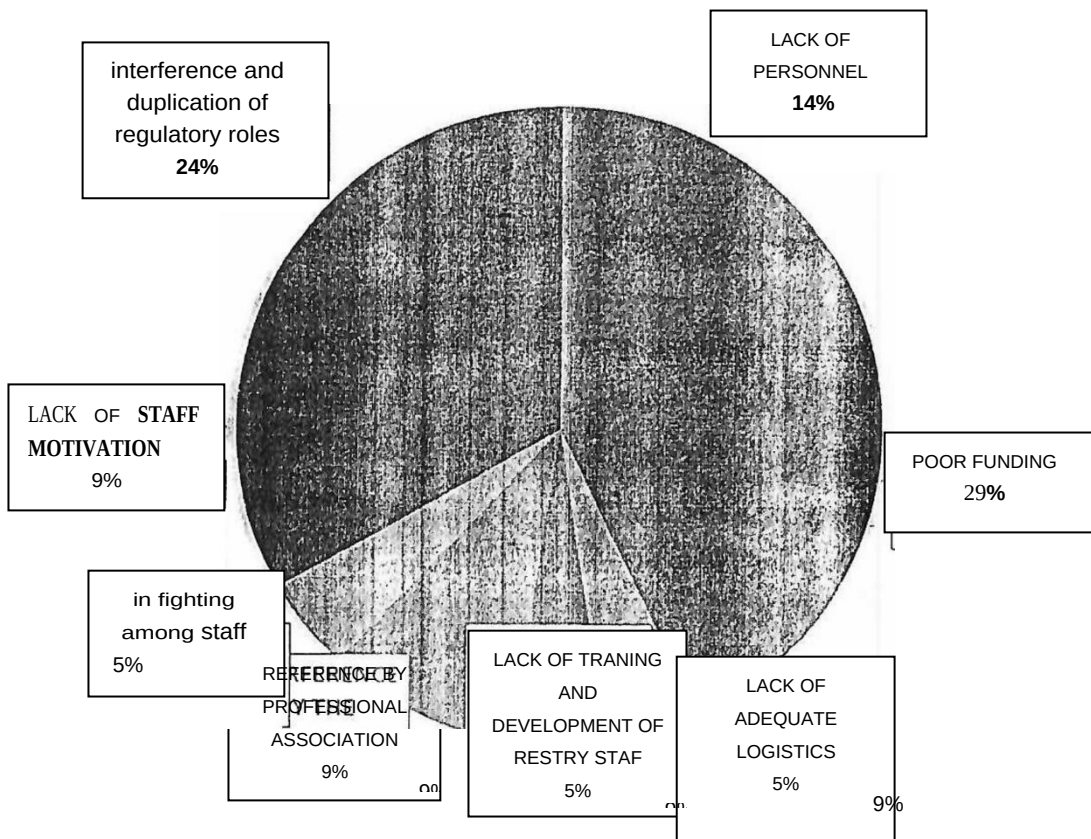
According to the respondent, the council often collaborates more with itself, Dental Therapist Registration Board of Nigeria (DTRBN) and Medical Laboratory Science Council of Nigeria (MLSCN) respectively among others.

Interpretation:

(i) 17% DTRBN;

- (ii) 13% MLSCN;
- (iii) 11 % PCN.

3.3 (d) Percentage Rating of the Following Challenges Faced by the Council



Interpretation

The challenges faced by the Council (with their percentage ratings) are:

- i. 14% Lack of personnel
- ii. 29% Poor funding
- iii. 24% Interference and the duplication of regulatory roles

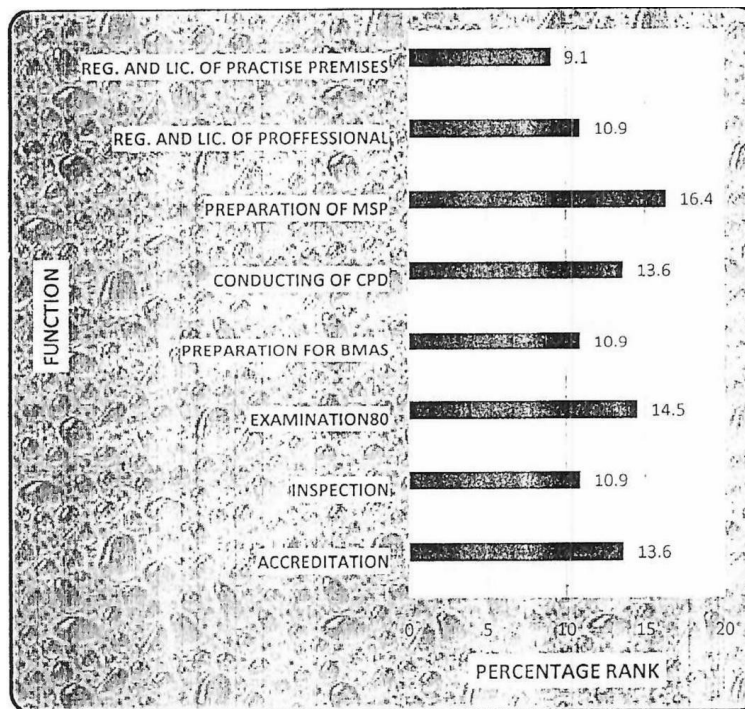
Hence, we can conclude that the Council's major challenges are poor

funding, interference and duplication of regulatory roles and lack of personnel respectively, among others.

4.0 THE DENTAL TECHNOLOGISTS' REGISTRATION BOARD OF NIGERIA (DTRBN)

The above Board was established in 1987. The establishment was applicable to Decree 43 of 1987 now Cap d6 of 2004 Abuja. The Board has total staff strength of 85.

4.4(a) Percentage Rank of the Board's Responsibility to their Functions



Interpretation

According to the respondent rating, the Board is responsible for

- i. 13.6% accreditation.
- ii. 14.6% examination.
- iii. 16.4% preparation of benchmark minimum academic

standards (BMAS)

1v. 13.6% conduction of continuing professional development (CPD) programmes

Hence, the Board performs majorly the functions of the registration and licensing of professionals, the examination, and accreditation, conduct of continuing professional development (CPD) and the preparation of minimum standards of practice respectively, among others.

4.4(b) Percentage Rank of Places where the Board's Regulatory Activities Takes Place



Interpretation

According to the respondent rating, the Board is responsible for 13.6% accreditation.

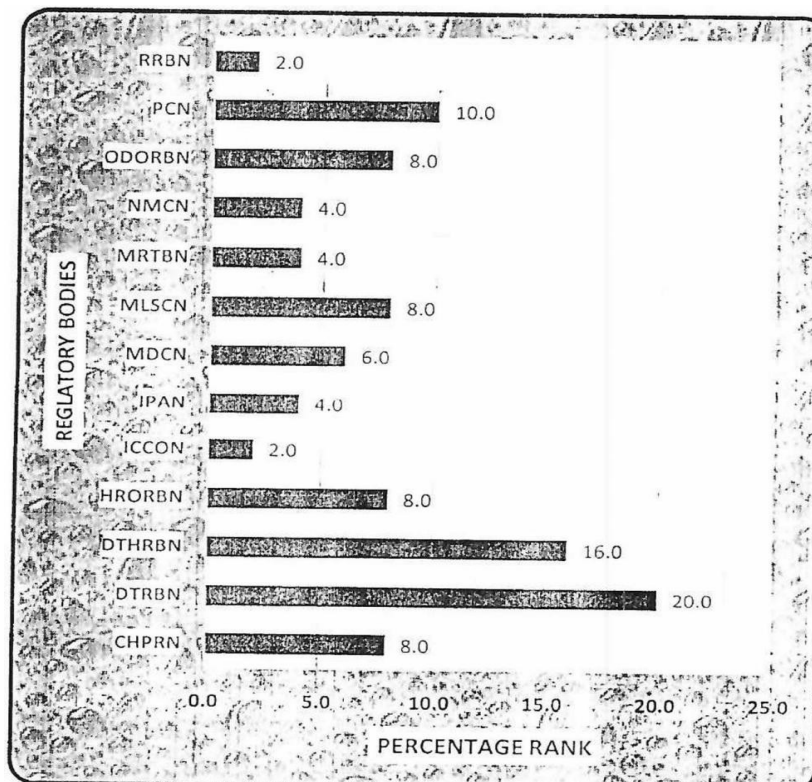
14.6% examination.

16.4% preparation of benchmark minimum academic standards (BMAS)

iv. 13.6% conduction of continuing professional development (CPD) programmes

Hence, the Board performs majorly the functions of the registration and licensing of professionals, the examination, and accreditation, conduct of continuing professional development (CPD) and the preparation of minimum standards of practice respectively, among others.

4.4(b) Percentage Rank of Places where the Board's Regulatory Activities Takes Place



According to the respondent rating, the Board regulatory functions take place in the following places (with percentages frequency)

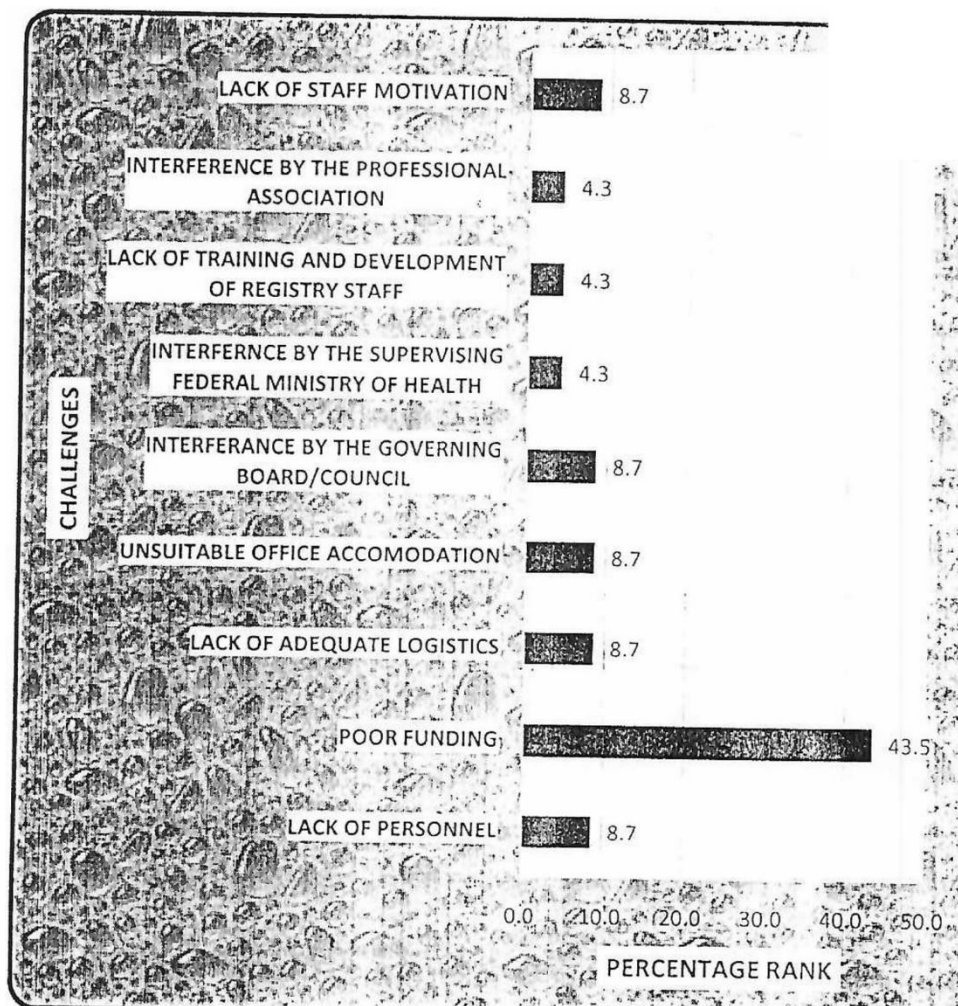
20% on University grounds;

20% on tertiary health care facilities

12% on secondary health care facilities

Hence, the Board uses tertiary institutions, University and the institute for the training of the regulated professional cadre majorly for their regulatory functions.

4.4{d) Percentage Rating of the Following Challenges Faced By the Board



Interpretation

The challenges faced by the Board (with their percentage ratings) are:

- (i) 8.7% Lack of personnel
- (ii) 43.5% Poor funding
- (iii) 8.7% Lack of adequate logistics
- (iv) 8.7% Unsuitable office accommodation
- (v) 8.7% Interference by the Governing Board
- (vi) 8.7% Lack of staff motivation.

Hence, we can conclude that the Board's major challenges are poor funding, lack of adequate logistics, unsuitable office accommodation and lack of staff motivation respectively, among others.

5.0 Conclusion, Recommendations and Implementation strategies

5.1 Conclusion:

(A) HRORBN:

The responsibility of the Board is high on Examinations (32%) and Accreditation (23%) while it is low on conducting CPD (2%). Indeed, in Registration and the licensing of practices and capacity building programmes for staff, it is 2% for each.

In places where the Board's Regulatory function takes places, SHT and Institution-for the training of the regulated professional cadre are high with 33% and 29%, respectively. It is low in university (2%) and Missionary Health Institution (2%) while MoHs and Private practice are equally low with 2% each.

In dealing with sister regulatory bodies, HRORBN has more dealings with MRTBN (12%) and CHPRN (10%) but low with DTRBN (2%) and ICCON (2%). It has no relationship with RRBN (0%).

On the challenges faced by HRORBON, poor funding is high (24%) as with unsuitable office accommodation (19%). Lack of personal is at 5%. Interferences are very low with FMOH at (5%).

(B) PCN:

The responsibility of PCN is high on Accreditation (11%); Inspection (11%); Preparation of BMAS (11%); Conducting CPD (11%), Registration and Licensing Professionals (11%) and Registration and Licensing practice premises (11 %). It is only low on examination (6.6%).

Places where the Council's regulatory functions often take place are high at university (13.7%); Tertiary healthcare facilities (12.7%); Secondary healthcare facilities (9.8%); Private Practice (11.8%); SHTS (11.8%); Institutions for the regulation of the regulated professional Cadre (13.7%); Missionary Health Institution (11.8%); and Ministries of Health (10.8%). It is only low at PHCs (3.9%).

In collaborating with sister regulatory bodies, PCN scored 100% high for most of the other twelve (12) regulatory bodies, possibly due to presence and necessity of drug products in each of the agencies for them to function effectively.

On the challenges, PCN indicated lack of adequate logistics (25%), poor funding (22.2%) and lack of personnel (19.4%), among the highs. Lack of staff motivation (2.8%) is the only parameter with a single digit as a challenge. In-fighting among registry staff is 0%.

(C) MDCN:

The responsibilities of the MDCN are high on registration and licensing professionals (22%), preparations of MSP (17%), Preparation of a code of conduct for the profession (17%) and Accreditation (17%). It is low on each of Inspection (5%), Examination (5%), preparation of BMAS (5%), conducting CPD (6%) and capacity building programme for staff (6%).

In places where the MDCN's regulatory work takes places, they are high in university (17%), and Institutions facilities for the training of the regulated

professional cadre (22%). It is low on Missionary Health Institution (6%) and Primary Health Care Facilities (5%). On collaboration with other health regulatory agencies, MDCN has more to do with DTRB (17%). MLSCN (13%) and PCN (11 %). It is low on collaboration with MRTBN, IPAN, ICCON, RRBN at 2% in each case.

On the challenges facing the MDCN, it indicated poor funding (29%) and Interference/duplication of regulatory role (24%) among the highs while in fighting among registry staff (5%) is low, as well as lack of logistics (5%). Indeed, no challenges are faced in respect of unsuitable office accommodation (0%), interference by the Governing Council (0%) and the supervising FMOH (0%).

(D) DTRBN:

The responsibility of the Board is high on registration and licensing professionals (10.9%); Examination (14.6%) and Accreditation (13.6%), conducting CPD (13.6%) and the preparation of MSP (16.4%).

It is low on capacity building programme for staff (5%) and none in the preparation of a code of conduct for professionals (0%) and discipline for members (0%).

In places where the Board's regulatory function takes place, we have University (20%); tertiary healthcare facilities (20%) and institutions for the training of the regulated professional cadre (20%) among the highs. Missionary health institution (4%) and MOHS (4%) are low, with no presence at SHT (0%).

In dealing with sister regulatory bodies, DTRBN claimed, to have high relationship with DTHRBN (16.0%), MLSCN (8.0%) and PCN (10%). It is low with nurses at 4. % and similar percentage with MRTBN, IPAN and very low with RRBN at 2%.

On the challenges faced by DTRBN, it indicates poor funding (43.5%) as the most critical. Interference by Governing Council (8. 7%), lack of motivation (8. 7%) and lack of personnel is 8. 7%. with professional association it rated the challenge at 4.3%.

5.2 Recommendations:

(a) Recommendation Number 1: HRORBN needs to address the following:

- (i) Regulating the activities of the professionals and practice areas;
- (ii) Conducting CPD programmes;
- (iii) Inspection of premises;
- (iv) Preparations of BMAS;
- (V) Registration and licensing Professionals;
- (vi) Should be given mandate to close down both government and private facilities that do not comply with the minimum standards.

(b) Recommendation Number 2:

PCN Needs to address the following: -

Setting and enforcing of standards that would promote safe and effective healthcare delivery in a harmonious atmosphere that recognizes and respects the boundaries of all the players in the team;

Provision of adequate pharmaceutical inspectors and inspection vehicles in all its offices.

(c) Recommendation Number 3:

MDCN needs to address the following:

- (i) Maintain and improve standards of practice among professionals;
- (ii) Eliminate quackery in the professions;
- (iii) Improve on the quality of training of professionals;

- (iv) Improve on the regulation / registration of professionals;
- (v) Improve on the quality of patient care.
- (d) Recommendation Number 4:

DTRBN needs to address the following

- (i) Inspection;
- (ii) Preparation of BMAS;
- (iii) Registration and licensing of practice premises;
- (iv) Capacity building programmes for staff.

5.3 Implementation strategies:

The members of CRHP must continue to collaborate with each other to peer review their regulatory and control functions, especially in critical areas of inspections, accreditations, registration & licensing, CPD programmes and in setting minimum standards of practice. There are no two ways about these in regulating and controlling their various profession's education and practices.

Similarly, all the regulatory bodies, singly and as CRHP members should establish linkages with international bodies like the World Health Professional Alliance (WHPA) and partner at the global level towards imbibing the best global practices in the dispensation of regulatory and control services in Nigeria in a most corrective and ethical manner as public institutions.

In the final analysis, it is the resolution of the WHPA that should prevail

and which emphasized that facilitating collaboration between key health professionals and major international stakeholders such as Governments, Policy makers and the World Health Organization (WHO). That by working

in collaboration instead of along parallel tracks, patients and the Healthcare systems of our world would benefit tremendously.

Thank you.

Dr. Ahmed T. Mora

Las Vegas, Nevada,

June 23-26, 2016

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Ibrahim A.A. (2016: Address by Dr. A.A Ibrahim, the Registrar, MDCN

while deliver the Keynote on "Minimum Standards Required in tertiary Healthcare Facilities for effective Service Delivery" at the Banquet Hall of Hotel Seventeen Lafiya Road, Kaduna on Wednesday, 13" March, 2016

International Journal of Healthcare Management, (Vol. 1 No. 1); December, 2015 P 36- 57.

**"THE TSAMIYA PHENOMENON: TRIBUTE TO MALLAM DAHIRU
SULEIMAN WALI ON ATTAINING THE GREAT AGE OF 81"**

**Printed in the Leadership Newspaper Edition of Saturday, July 23,
2016on Page 50, _Under Tribute**

By

Dr. Ahmed T. Mora, _mni

In pharmacy circle in Nigeria, the names, Mallam, or Pharmacist D.S. or Tsamiya, all refer to one person, and that person is Pharmacist Dahiru

Suleiman Wali, son of the first Walin Kano, Suleiman (1890-1939), grandson of Isma'il, Limamin Kano, and great grandson of Suleiman, Sarkin Ruru and who is eighty-one (81) years old today. This tribute about a great and thoroughbred professional Pharmacist D.S. Wali on his eighty first (18st) birthday anniversary today is deserving one for a legend of our time as one or the finest in the profession of pharmacy. According to His Excellency, Alhaji (Dr.) Yusuf Maitama Sule, CFR, Danmasanin Kano, and one time teacher of D.S. Wali - (my relationship with Dahiru D.S. Wali) is a very special one. With Sheikh Na'ibi Suleiman Wali (now deceased) and the late Isa Wali, both senior brothers of Dahiru, we have been friends and played together as children in Kano. Dahiru was my student when I was a teacher in Kano Middle School. Over the years, Dahiru became my friend, a very close and sincere friendship developed between us. We never played with one another but showed only sincerity and true friendship.

Mallam Dahiru Suleiman Wali, popularly known as D.S. Wali, is a pharmacist by training, practice and conduct. He epitomizes professional excellence in his Tsamiya Pharmaceutical Company Limited whose doors opened to the public along Ahmadu Bello Way, Kaduna in 1975. During the twenty-eight (28) years that Mallam D.S. Wali practiced as a Community Pharmacist at Tsamiya Pharmaceutical Company Limited in Kaduna, he redefined the way and manner Community Pharmacy should be practiced in the best of standards and in conformity to the Code of Ethics of the Pharmacy Profession.

By 2003, when Mallam D.S. Wali retired voluntarily from his own practice and closed the doors of his Tsamiya Pharmaceutical Company then at Yoruba Road, Kaduna, he had personified best practices in the specialty of Community Pharmacy practice in Kaduna and Nigeria. Little wonder that till date, professional colleagues and/or patients still refer to him as Mallam Tsamiya or simply Tsamiya. To achieve this label signifying practice, branding and incorporation is very rare among community pharmacists or indeed other practitioners of the profession in Nigeria.

Prior to the establishment of Tsamiya Pharmaceutical Company Limited Mallam D.S. Wali had practiced as a hospital and industrial pharmacist in

such places as Ilorin, Sida, Kaduna, Zaria and Lagos in Nigeria and in faraway United Kingdom (UK) between the years 1957 and 1975. In each case, Mallam D.S. Wali had distinguished himself as a professional pharmacist per excellence.

Mallam D.S. Wali is retired from practice of the pharmacy profession Community, Industrial or Hospital. He is now simply leading the life of a retired pharmacist, having served Nigeria and indeed the health-care profession of pharmacy at public and private capabilities for forty-six (46) years (1957-2003). What more would a person want? He is enjoying his life in retirement with his amiable wife of fifty-two (52) year, Jutta, in their beautiful residence on 4A, Dala Road, Government Reservation Area (GRA) in Kaduna. The couple's son Dr. Muhammad Ujudud (Andreas), is in the USA working as a specialist physician who has specialized in intervention cardiology. Dr. Wali, regards his father, the senior Wali, as his role model. Hajiya Aisha (Melanie) Dikko, the second and last child of the Wali's is happily married to Alhaji Ibrahim Dikko and lives in Lagos with her husband. Aisha (Melanie) described her father as a morally upright and kind person. Both Dr. Ujudud and Aisha have given the Wali's four beautiful grandchildren Jaydaa, Amar, Tristan and Sedrick. The Wali's are a happy family. Life-has been good to them, devoid of any controversy. As concluded by Judda, we seem to have reached still waters, the boat glides smoothly along. The life and times of Mallam D.S. Wali are certainly that of a case study of a dedicated and professional pharmacist who came, saw and conquered. The history of pharmacy practice in Nigeria especially that of Community Practice, would never be complete without mention of the name of Mallam D.S. Wali in gold.

The community practice of Mallam D.S. Wali largely revolved around Kaduna, the capital of the former Northern Region of Nigeria and the old Kaduna State (now Kaduna and Katsina State) and the present Kaduna State. He made Kaduna his home in every sense of belonging. He reached out to the academic section of the Pharmacy profession through his frequent intervention in the Faculty of Pharmaceutical Sciences, Ahmadu Bello University (ABU), Zaria as a member of the Faculty Board and even beyond. He bought textbooks on aspects of pharmacy and the

pharmaceutical sciences, donated money unsolicited for in cash or by cheque all in an attempt to promote teaching, learning and research in pharmacy at ABU and generally Nigeria.

This was and still is quintessential Mallam D.S. Wali, ever so willing to give and not wanting to make it public. His nephew, Alhaji Salahuddeen Na'ibi Wali, confirmed this when in an interview, he stated that Mallam D.S. Wali had for many years practically placed his sisters on monthly monetary allowances. These allowances were "specially", sent to the sisters through Salahuddeen who made sure that the amounts were distributed promptly as directed by his uncle, Mallam D.S. Wali.

I am a living witness of such generosity at the Faculty of Pharmaceutical Sciences, ABU, Zaria after a group of Pharmacy graduates from the faculty were inducted and administered the Pharmacist's Oath on February 7, 2011. Mallam D.S. Wali brought out a cheque of Fifty Thousand Naira (N50,000.00) and handed it over to the Dean of the Faculty as a contribution to the faculty- completely unsolicited. This was the generosity of a retired pharmacist. Mallam D.S. Wali had voluntarily retired from active pharmacy practice in year 2003.

On Saturday, 6th April, 2013 and at the Mambayya House, Bayero University Kano (BUK), the book- "D.S. Wali: An Icon of Community Pharmacy Practice in Nigeria" authored by myself, Professor Umar Usman Pateh; Professor Ibrahim Adamu Yakasai, Pharm. Aliyu Jibrin and Pharm. Ahmed Garba Gana Mohammed was presented to the Nigerian public. To date, the book remained the first biography of any pharmacist living or deceased in the Northern part of Nigeria and the second in the whole Country after that of the late Dr. Fred Adenika (1940 - 2003).

A common thread that binds the authors of this book with Mallam D.S. Wali is the fact that they all considered Mallam D.S. Wali as a father professionally and traditionally in the true customs of Northern Nigeria. Mallam D.S. Wali had intervened at one time or the other during the careers of the authors by ways of advice, guidance and/or counseling.

It is this direction given to virtually all the authors by Mallam D.S. Wali as well as his personal life-style and example that further endeared him to the authors. Hence, their efforts in putting the book together so that other pharmacists and present-day Nigerians and those yet unborn can similarly tap from the experience of this icon of Community Pharmacy Practice in Nigeria.

It was therefore, little wonder that on 23rd July, 2015, the eightieth (80)

birthday anniversary of the pharmacy practice guru, the members of the Pharm. D.S. Wali Book Project Committee celebrated it with the 1st Pharm. D.S. Wali, FPSN Lecture delivered by Professor Umar Usman Pateh, a member of the committee and the Dean, Faculty of Pharmaceutical Science Ahmadu Bello University (ABU), Zaria. Dr. Stephen C. Kitchener, a Medical Practitioner of repute and the MD and proprietor of Anna-Kitch Medical Centre, Wusasa, Zaria Chaired the lecture while Pharm. Sam Nda Isaiah (Kakakin Nupe) and Chairman, Leadership Group was Special Guest. As it was in 2015, so shall it be in 2016. The Committee is celebrating the 81st birthday of its champion with a 2nd Pharm. D.S. Wali, FPSN, Lecture to be delivered by Professor Haruna Abdu Kaita, a distinguished pharmacist and the new Vice Chancellor, Federal University, Dutsinma, Katsina State with title- Food Additives and Household Chemical Exposure: Pharmaceutical Information, Education and Counseling.

The immediate former Vice Chancellor, ABU, Zaria and a pharmacist of repute, Professor Abdullahi Mustapha, FPSN will be the Chairman at the Lecture which will take place today at the Arewa House, Kaduna from 9:00am. We wish Mallam D.S. Wali a happy 81st birthday anniversary and wishing him many happy returns in excellent health, economic prosperity and added wisdom, Amin.

Happy Birthday Anniversary, sir.

Dr. Ahmed Mora is a Pharmacist and the *Wakilin Maganin Zazzau* and writes from Kaduna.